

# Clinical features and treatment of peristomal pyoderma gangrenosum.

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## Evidence abstract

**CONTEXT** Peristomal pyoderma gangrenosum (PPG), an unusual variant of pyoderma gangrenosum, has been reported almost exclusively in patients with inflammatory bowel disease (IBD) and is frequently misdiagnosed.

**OBJECTIVE** To better characterize the clinical manifestations, diagnosis, and management of PPG.

**DESIGN, SETTING, AND PATIENTS** Retrospective analysis of 7 patients with PPG observed in a university-affiliated community setting between 1988 and December 1999.

**MAIN OUTCOME MEASURES** Clinical and histopathologic features, associated disorders, and microbiologic findings.

**RESULTS** Two patients had Crohn disease, 2 had ulcerative colitis, and 3 had abdominal cancer.

Five patients had at least 1 relapse of PPG after initial healing.

Although 3 of 4 patients with IBD had active bowel disease, a parallel course with PPG occurred in only 1 patient.

Both patients whose stoma was relocated developed an ulcer at the new site.

Effective therapies included topical superpotent corticosteroids; intralesional injection of triamcinolone acetonide at the ulcer margin; topical cromolyn sodium; oral dapsone, prednisone, cyclosporine, mycophenolate mofetil; and intravenous infliximab.

**CONCLUSION** Our experiences demonstrate that although PPG has been most often reported in patients with IBD, it may occur in the absence of IBD.

Biopsy of the skin lesion is not diagnostic but excludes other causes.

Relocation of the stoma may be associated with a new ulceration and should be avoided.

Trauma to the skin of a predisposed patient may elicit the pustules or ulcerations associated with pathergy.

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